PMCF Data Collection Workflow

This document summarizes the structured post-market clinical follow-up (PMCF) workflow used for the collection, validation, storage, and analysis of real-world clinical data from patients with fibromyalgia treated with low-intensity repetitive transcranial magnetic stimulation (Li-rTMS).

1. PMCF Framework

The PMCF program was implemented across multiple outpatient centers using a standardized Google Forms-based clinical questionnaire. The workflow included longitudinal assessments performed during predefined treatment stages:

- Baseline assessment (before first session)
- Intermediate assessment (approximately 4th session)
- Final treatment assessment (approximately 8th session)
- Follow-up / recall sessions upon patient request

2. Questionnaire Workflow

Step	Description
1	The nurse or clinical assistant initiated the questionnaire and entered center and physician information.
2	The patient completed the questionnaire before the corresponding treatment session.
3	The questionnaire included WPI, FIQ, SS Score, and predefined treatment assessment items.
4	The last section of the questionnaire corresponded to the predefined Treatment Assessment component of the PMCF questionnaire and was administered only during intermediate, final-treatment, and follow-up/recall phases. These items assessed patient-reported headache perception, generalized pain perception, and sleep-related symptom perception during Li-rTMS exposure.
5	Completed questionnaires were automatically sent to the responsible treating physician.
6	The physician reviewed and validated questionnaire responses before forwarding them to the centralized PMCF database.
7	Data were stored in anonymized spreadsheet format for descriptive

	statistical analysis and PMCF surveillance activities.

3. Data Governance and Validation

Patient questionnaires were reviewed and validated by the responsible treating physician before inclusion in the centralized PMCF database. The validation process was intended to ensure completeness, consistency, and appropriate clinical supervision of the collected responses.

The publicly shared repository dataset is fully anonymized and does not contain direct patient identifiers, clinical history numbers, exact dates, or identifiable center information.